

Frequently Asked Grant Application Questions

1. Problem Statement

What problem are you solving? Why is this problem important?

What funders are really asking:

Is this a real, urgent, well-understood problem or just an idea?

How to answer (structure):

Start with a clear, specific problem

Add data/statistics (local + global if possible)

Show who is affected and why

Highlight the gap in existing solutions

Template:

[Target population] are facing [specific problem], resulting in [consequence].

In [location], this affects approximately [data/stat].

Existing solutions such as [X] fail because [gap].

This creates an urgent need for [your solution type].

Pro tip:

Avoid general statements like “lack of access.” Define what kind of access, where, and why it fails.

Paper-based health records remain a major challenge across Nigerian healthcare facilities. In fact, about 82% of Primary Healthcare Centers (PHCs) and private clinics still rely on paper records for patient data management. To put this into a tangible view, nearly 70% of Nigerians depend on PHCs and private clinics for healthcare services, and these facilities account for approximately 85.3% of healthcare institutions in the country (BMJ Global Health, 2025). The challenges of paper records extend beyond healthcare delivery alone. In 2020, AI engineers at Robotics and Artificial Intelligence Nigeria could not train a diagnostic model because medical records were paper-based, fragmented, and unsearchable (Tech Safari, 2026).

Nigeria, being the most populous country in Africa, with an estimated population of 220 million people where Lifeven Health is located in the city of Lagos. Lifeven Health is an Health-tech startup incorporated in 2026 with a mission to improve care outcomes, and provide care continuity through an affordable digital electronic health record system for low-to-medium healthcare facilities in Nigeria.

Even though Lifeven Health is situated in Africa's largest population center, Nigeria continues to experience a high mortality rate associated with discontinuous care. This is often caused by incomplete patient health records, delayed diagnosis, repeated medical tests, medication errors, interrupted treatment plans, and poor long-term patient monitoring. These challenges have contributed to poor patient data accessibility, weak disease surveillance systems, poor public health planning, and the continued dependence on paper-based health records, where an estimated 30–40% of patient files are lost or misfiled annually (Nigeria Demographic and Health Survey, 2023).

Statistics provided by the World Health Organization (WHO) shows over 95 million Nigerians are living with the dual burden of communicable and non-communicable diseases (NCDs), with the latter making up 27% of deaths in 2019 (WHO, 2023). In a more explanatory view, the age-standardized mortality rate for the four major NCDs (cardiovascular disease, chronic respiratory disease, cancer, and diabetes) reached 565 per 100,000 in males and 546 per 100,000 in females as of 2021. At the same time, communicable diseases such as malaria remain a significant public health problem with an estimated 65.4 million cases in 2021, resulting in 193,512 deaths (WHO 2023).

Lifeven Health recognizes the challenge of interoperability health data sharing across healthcare facilities in Nigeria. However, fragmented paper-based records and discontinuous care remain the primary problems that need to be addressed. Across many low-to-medium healthcare facilities in Nigeria, patients' health records are still paper-based, manually stored, and often inaccessible across different points of care. Although some digital health systems exist within larger private hospitals and big government owned healthcare institutions, they are often unaffordable, inaccessible, or inadequately designed for small and medium-sized healthcare facilities that serve the majority of Nigerians.

Therefore, our problem statement is: In low-to-medium Nigeria healthcare facilities there are no digital health record systems for continuous patients' care. This void of digital health record system contributes to fragmented patient information, poor continuity of care, weak disease surveillance systems, delayed diagnosis, repeated medical tests, medication errors, treatment setbacks, and poor public health planning, and increased mortality rates.

Inspiration

My inspiration for building Lifeven Health came from recognizing that when paper health records cost lives, the question is no longer whether the system is failing people, but how many lives are being affected daily by fragmented healthcare data.

Meet Ibrahim Zira, a 64 years old patient living with high blood pressure, and managing the condition at Jigalambu Primary Healthcare Centre (PHC) in the Michika area of Adamawa State, northeastern Nigeria. When his condition worsened, he was referred to the Michika General Hospital, where he faced a familiar struggle: incomplete medical records. "Ibrahim said, when I got there, they asked for my health records, and the file I had contained very little information. I was asked questions and told to repeat tests. I had to pay again. It was painful because I don't have a steady income," This inspired me to build a system that improves health data accessibility, interoperability, and continuity of care for underserved healthcare facilities.

2. Your Solution

What is your solution? How does it work?

What funders are really asking:

Is this clear, practical, and different enough to matter?

How to answer:

Describe your solution in one clear sentence

Break down how it works step-by-step

Emphasize what makes it unique

Template:

We provide a [solution] that enables [target group] to [key benefit].

The solution works by:

[Step 1]

[Step 2]

[Step 3]

Unlike existing solutions, we [key differentiator].

Pro tip:

If a 15-year-old can't understand your solution, a grant reviewer won't either.

Lifeven Health is a cloud-based, mobile-first, multi-tenant digital electronic health record (EHR) system with offline capability designed specifically for low-to-medium Nigeria healthcare facilities including Primary Healthcare Centers (PHCs), private clinics, and diagnostic centers. The platform is focused on improving patient care outcomes, reducing discontinuous care, health data interoperability, and strengthening healthcare delivery for thousands of facilities providing care to millions of Nigerians

The solution works through three core operational components:

Point-of-Care Digitization:

Clinicians working in the 85.3% of Nigerian healthcare facilities that still rely on paper records can instantly capture patient information using a mobile-first application. Since many facilities experience unstable internet connectivity and power supply, the platform is designed with offline-first functionality. Patient records are securely stored locally on the device and automatically synchronized to the cloud once internet connectivity becomes available. This ensures uninterrupted clinical workflows regardless of infrastructure limitations.

Unified Patient Identity and Interoperability:

By integrating the National Identity Number (NIN), the platform assigns a unique digital identity to every patient, enabling consistent and interoperable medical records across multiple healthcare facilities within the network. This approach significantly reduces the estimated 30–40% patient file loss and misfiling rate associated with paper-based systems. As a result, clinicians can instantly retrieve a patient's medical history, reducing repeated diagnostic tests, medication duplication, and treatment interruptions caused by missing records.

Real-Time Health Intelligence and Surveillance:

The platform converts fragmented clinical records into structured and actionable healthcare intelligence. Clinics and healthcare providers can monitor disease patterns, track treatment outcomes, and strengthen public health reporting in real time. This capability is particularly important in addressing the growing burden of non-communicable diseases, which account for approximately 27% of deaths in Nigeria, while also improving surveillance and response efforts for communicable diseases such as malaria, which recorded an estimated 65.4 million cases in 2021.

What differentiates Lifeven Health from existing electronic health record solutions is its infrastructure-aware design for low-to-medium resource healthcare facilities. Existing enterprise EHR systems are expensive, internet-dependent, and designed primarily for large urban hospitals with stable internet and power supply. These limitations have contributed to the low digital adoption rate across smaller facilities.

Lifeven Health is specifically built for "systems for health" and to bridge the gap between advanced healthcare technology and the operational realities of underserved healthcare facilities. Through an affordability, offline capability, interoperability, and mobile accessibility into a single solution tailored for low-to-medium healthcare facilities.

3. Innovation

What makes your solution innovative?

What funders are really asking:

Why should we fund this instead of 100 similar ideas?

How to answer:

Frame innovation in one of these ways:

New delivery model

New audience reach

New combination of tools

New cost structure

Template:

Our innovation lies in [specific innovation].

While [existing approach] focuses on [X], we improve this by [your improvement].

This allows us to achieve [better outcome: cheaper, faster, wider reach, etc.].

Pro tip:

Do NOT say "we are the first." Say why your approach works better.

Our innovation combines the implementation of an offline-first synchronization approach, with a National Identity Number(NIN) linkage for reducing duplicate patient data, a queue-based monitoring optimizes clinic operations and reduces patient wait times, integration of OCR AI vision for evaluating patients' past medical records into the new digital system, and an

interoperability medical data sharing layer between private clinics and diagnostic centers for patient care continuity.

While existing EHRs focus on digitizing individual health facilities records, we improve this by using global health standards for interoperability, creating an openEHR canonical data modeling and HL7 FHIR for secure longitudinal data sharing, we transform fragmented and isolated medical records into a standardized, interoperable ecosystem where data is both portable and secure.

This allows us to achieve total care continuity, eliminates duplicate patient entries, improves operational efficiency, significantly reduces the financial burden of repeated diagnostic tests by providing continuous care for **219 million people**, which lays the foundation for a secure longitudinal data sharing for healthcare facilities in Nigeria.

4. Target Beneficiaries

Who are your beneficiaries? Who does this solution serve?

What funders are really asking:

Is this focused, and does it prioritize the right people?

How to answer:

Be specific and narrow

Include demographics + context

Show why they are underserved

Template:

Our primary beneficiaries are [specific group: age, gender, income level, location].

These individuals currently face [key barrier].

We prioritize this group because [reason: vulnerability, gap, impact potential].

Pro tip:

“Women and youth” is too broad. Narrow it down.

Our primary beneficiaries are small-to-medium private clinics and diagnostic centers in urban and peri-urban Nigeria, and by extension, the diverse patient population they serve living with the burden of Communicable and Non-communicable diseases.

These healthcare facilities currently face a massive financial barrier in adopting enterprise EHRs, leaving 85.3% of them reliant on paper records. This creates a "data vacuum" where clinics lose and misfile 30% to 40% medical records annually (Nigeria Demographic and Health Survey, Year).

We prioritize this group because they are the frontline of defense against Nigeria's dual burden of disease. Nigeria currently accounts for **30% of worldwide malaria deaths**, with **65.4**

million cases annually(WHO 2023), yet there is "limited data" to track treatment outcomes effectively. By empowering these specific groups with affordable, offline-capable digital tools, we fill a critical surveillance gap and provide a "digital birthright" of health records for chronic disease patients who are currently underserved by the fragmented paper-based system.

5. Impact & Outcomes

What impact will your solution create? What are your expected outcomes?

What funders are really asking:

What exactly will change—and can you measure it?

How to answer:

Use numbers + timeframes

Focus on outcomes, not just activities

Template:

In the next [timeframe], we will:

Reach [X] beneficiaries

Achieve [specific outcome, e.g., increase ANC visits by 40%]

Reduce [problem] by [percentage or number]

Long-term, this will contribute to [broader impact].

Pro tip:

If you can't measure it, funders won't trust it.

In the next 12 months, we will:

Onboard 1 private clinic and 2 diagnostic centers to pilot our first software solution rollout, establishing an interoperability digital health footprint in a sector where 85.3% of facilities still rely on paper records.

Achieve a 70% rate in digital record sharing within these facilities and maintain a 95% confidence score in OCR-driven document evaluation to ensure high-fidelity data transition from legacy paper files.

Reduce average patient waiting times by 15% through our integrated queue-based monitoring system, while simultaneously decreasing duplicate patient records and care discontinuity by 40%.

Long-term, this will contribute to the Nigeria Digital Health Architecture (NDHA) for an Interoperable Digital Health Ecosystem by putting patients at the center, advancing healthcare equity and access, and building a system where technology amplifies the compassion and expertise of Nigerian citizens.

6. Traction / Validation

What progress have you made so far? Do you have any results or pilots?

What funders are really asking:

Is this tested or just theoretical?

How to answer:

Share real evidence

Even small wins matter

Template:

So far, we have:

[Piloted/tested solution with X users]

[Achieved result: e.g., 80% user satisfaction]

[Partnerships, endorsements, or early adopters]

Pro tip:

No traction? Then show strong research or prototypes.

So far, we have:

Developed a cloud-based Minimum Viable Product (MVP) that has undergone rigorous technical validation, confirming the feasibility of our interoperable architecture and its strict alignment with global health data standards. This MVP achieved premier industry recognition by winning 1st place at the 2025 Codemantic Build with Google Hackathon, a milestone that validates our technical framework and its potential to solve complex data fragmentation in healthcare.

Currently, we enrolled in the Africa Health-Tech Accelerator Program, providing us with the strategic mentorship and institutional support necessary to transition from a validated prototype to a market-ready solution for the Nigerian sector within the next 6 months.

7. Scalability

How will you scale this solution?

What funders are really asking:

Can this go beyond a pilot and reach thousands/millions?

How to answer:

Show replication model

Show distribution channels

Show cost logic

Template:

Our solution is scalable because it leverages [system/channel].

We plan to expand by [strategy: partnerships, tech, community networks].

This allows us to grow from [X] users to [Y] users within [timeframe].

Pro tip:

Mention systems, not just ambition.

Our solution is scalable because it leverages a cloud-native, vendor-neutral, and modular architecture built on global openEHR and FHIR interoperability standards, which allows for rapid deployment across diverse healthcare facility sizes without the need for expensive on-site hardware.

We plan to expand by forming strategic partnerships with private healthcare associations and diagnostic center networks, while ensuring our data models remain strictly compliance with Nigeria Data Protection Act (2023), and aligned with the Nigeria Digital Health Architecture (NDHA) for an Interoperable Digital Health Ecosystem (2025) as published by the Nigeria Federal Ministry of Health & Social Welfare.

This allows us to grow from our initial 3 pilot facilities to a network of over 50 healthcare facilities across Nigeria within the next 24 months, creating a unified digital health ecosystem that can support hundreds of thousands of patient records.

8. Sustainability

How will your solution be sustained long-term?

What funders are really asking:

Will this die when our money runs out?

How to answer:

Show revenue streams or long-term funding strategy

Template:

We will sustain this solution through [revenue streams, partnerships, or funding model].

Over time, we aim to reduce reliance on grants by [strategy].

Pro tip:

Even nonprofits need a sustainability plan.

We will sustain this solution through a tiered B2B SaaS per-patient subscription model specifically designed for private clinics and diagnostic centers, ensuring that the cost of digital transformation remains proportional to facility size and patient volume.

Over time, we aim to reduce reliance on grants by converting our pilot facilities into long-term commercial partners by digitizing their entire operational flow, integrating patients' health insurance claims, and robust billing management tools that decrease denied claims and speed up collections.

9. SDG Alignment

Which SDGs does your solution address?

What funders are really asking:

Does this align with global priorities?

How to answer:

Be specific (Goal + Target)

Template:

Our solution aligns with SDG [number]: [goal name], specifically target [X], by [clear contribution].

Pro tip:

Tie SDGs directly to your metrics, not just mention them.

Our solution aligns with SDG 3: Good Health and Well-being, specifically targeting communicable and non-communicable diseases by reducing premature mortality rate by 15%.

Our solution aligns with SDG 9: Industry, Innovation, and Infrastructure, providing a resilient, digital health infrastructure for the 85.3% of Nigerian healthcare facilities currently reliant on paper records.

10. Budget & Use of Funds

How will you use the funding?

What funders are really asking:

Will this money be used effectively?

How to answer:

Be clear and realistic

Link spending to impact

Template:

The requested funding of [\$35,637] will be allocated as follows:

Pro tip:

Every dollar should connect to impact.

[\$130] – [Organization registration with Nigeria Corporate Affairs Commission (CAC)]
[\$77] – [Organization registration with Nigeria Data Protection Commission (NDPC)]
[\$0] – [Federal Competition and Consumer Protection Commission (FCCPC) & NITDA Startup Portal]
[\$200] – [Domain, SSL Certificate, and Cloud Hosting]
[\$140] – [OCR, and Identity Verification API Credits (NIN Linkage)]
[\$50] – [Trademark Registration]
[\$40] - [Logistics related expenses for pilot testing]
[\$35,000] - [12 months salary for a team of 4]

This investment will enable us to transition from an informal project to a legally recognized, compliant corporate entity. By securing these registrations, we mitigate the risk of regulatory fines (which can reach ₦10 million for data breaches) and build the "institutional trust" necessary to sign agreements with private clinics and diagnostic centers.

11. Team

Tell us about your team

What funders are really asking:

Can you actually execute this?

How to answer:

Highlight relevant experience

Show credibility + execution ability

Template:

Our team brings expertise in [fields].

[Founder] has [X years/experience].

Together, we have [key achievement or credibility].

Pro tip:

Focus on why you are the right team, not just resumes.

Our team brings expertise in full-stack software engineering, mission-critical backend architecture, and high-performance frontend interfaces, with a specific focus on the Nigerian HealthTech and FinTech ecosystems.

- Adeola Abdulramon (Founder) has 3 years of experience as a software engineer. Her dual experience in the HealthTech and FinTech sectors is critical for navigating the intersection of medical data integrity and the financial interoperability required for out-of-pocket payment systems.

- Joseph Ogunkunle (Senior Software Engineer): With 7 years of professional experience, Joseph provides the senior technical leadership necessary to manage our openEHR and HL7 FHIR data modeling. His expertise ensures that our "offline-first" architecture is resilient enough to handle the infrastructure challenges of 85.3% of Nigerian clinics.
- Delight Sheriff (Backend Developer): Delight specializes in robust server-side logic and database management. He is responsible for the NIN-linkage integration and the secure API layers that allow real-time data sharing between clinics and diagnostic centers.
- Daniel Fadamintan (Frontend Developer): Daniel focuses on creating intuitive, "low-friction" user interfaces. His work ensures that clinicians, many of whom are transitioning from paper for the first time, can navigate the digital system with minimal training, maximizing adoption at the point of care.

12. Why This, Why Now

Why is this the right time for your solution?

What funders are really asking:

Is this timely and urgent?

How to answer:

Link to trends, policy, or urgency

Template:

This is the right time because [trend, gap, or urgency].

Factors such as [X, Y, Z] make this solution especially relevant now.

This is the right time because Nigeria has reached a critical policy turning point with the launch of the **Nigeria Digital Health Architecture (NDHA)**, creating an urgent mandate for the private health sector to bridge the gap between paper-based records and a unified national ecosystem.

Factors such as the Ministry of Health's explicit call for private sector agility, the necessity of patient-centered interoperability to ensure care equity, and the technological readiness of cloud-native systems make our solution a vital catalyst for the nation's digital transformation. By aligning with the Minister's vision to amplify human expertise through technology, we are providing the essential infrastructure needed to move Nigeria's healthcare from fragmented, paper-heavy workflows into a vibrant, integrated future.

13. Mission and Vision

Why wecareEHR

Our mission is to improve care outcomes by streamlining the work of your care teams and accelerating Nigeria digital health transformation supported by systems for health. Using

international health standards, we build a personalised digital user experience on top of a vendor-neutral putting patients at the center, to advance equity and access.

14. Value Proposition

Reliable Point of Care Digitization for Healthcare Facilities

Sub-line

Small and medium healthcare facilities use wecareEHR online and offline to digitize patient paper records, secure data sharing, streamline clinical workflows, manage financial processes, enhance administration efficiency, and ultimately improve patient care outcomes.

15. Unfair Advantage:

Financial & Deployment Strategy

High upfront setup costs and complex implementations prevent hospitals from adopting EHRs.

- CapEx-Free/SaaS Pricing: Eliminate huge upfront licensing fees. Shift to a "Pay-As-You-Go" model or charge a low per-patient-visit fee, making it affordable for both small clinics and large hospitals.
- Hardware Provisioning: Partner with hardware leasing companies or telecom operators to bundle your EHR subscription with rugged, affordable tablets and solar-power backups, removing the hardware barrier entirely, even with data-subscription.

Partnership with regional healthcare facilities,

Partnership with hardware leasing companies and telcos operators for affordable tablets and solar-power backups

National Health Insurance Authority Integration,

Offline Capability, OCR Enabled, Unified Patient Identity, Interoperability, Data Compliance, Per-Patient Pricing Model

16. Theory of Change

Elevator Pitch:

Hi, I'm Adeola Abdulramon, a software engineer and founder of Lifeven Health.

When paper records cost lives, the question is no longer whether they are dangerous; it is why they persist.

In the narrow hallways of a public hospital in Nigeria, a woman endures labor through the night, gripping a worn antenatal card. Her blood type and previous complications are barely legible, yet that piece of paper is the only record connecting her past care to her urgent present. As clinicians try to reconstruct her medical history, precious minutes slip away.

This is not a rare exception—it is the daily reality in many Nigerian healthcare facilities, where 82% of hospitals still rely on paper records and over 95 millions of patients' lives depend on paper-based records that fade, tear, or vanish.

Witnessing challenges like these inspired me to build Lifeven Health, a mobile-first, offline-capable electronic health record system designed for low-to-medium healthcare facilities. Our goal is simple: point-of-care digitization for reducing fragmented paper records, preventing medical errors, treatment delays, care setbacks, and improving patient care outcomes.

I believe that a patient's medical history should follow them wherever they receive care, regardless of internet connectivity, infrastructure limitations, or location.

We are preparing to pilot Lifeven Health across 20 private hospitals in Nigeria within 6 months, and with your support we hope to help hospitals transition from fragmented paper-records to digital health systems to improve patients' care outcomes for millions of Nigerians.

Thank you.